

Purpose: To ensure correct and early identification, registration and continuity of care for patients with known or at-risk chronic conditions through standardized screening and documentation.

AT ALL STEPS: Protect dignity, privacy and confidentiality; use respectful, confidential and non-stigmatizing communication.

WELCOME, CONFIRM IDENTITY & SUPPORT NEEDS

- 1 Welcome the client and confirm identity (e.g., National ID). Obtain consent as appropriate and identify any disability or special-needs support required. **Records staff
CHW / support staff**

SEARCH FOR ANY EXISTING RECORDS BEFORE CREATING A NEW ONE

- 2 Search paper and current electronic/EMR records first to avoid duplication. For follow-up clients, retrieve the file a day before review where feasible. **Records staff
CHW / support staff**

**EXISTING PATIENT
RECORD FOUND?**

YES

NO

3A EXISTING CLIENT

Retrieve the record. Update locator, contact and next-of-kin/contact details at each visit as needed.

Records staff

NEW CLIENT

Register demographics & locator/contact details; assign a unique Clinic No.; issue a patient-held card/booklet enter in the current system.

Records staff

CONFIRM THE REASON(S) FOR TODAY'S VISIT

- 4 New illness, review, routine refill, missed appointment, or return after loss to follow-up. **Records staff**

SCREEN FOR PRIORITY & OUTBREAK-PRONE CONDITIONS

- 5 Screen for TB, HIV testing eligibility, HTN/DM, CVD risk, asthma and cancer, and for outbreak-prone diseases e.g., VHF. **Nurse / Counsellor**

MEASURE & RECORD VITALS / ANTHROPOMETRY

- 6 Using calibrated equipment, record temperature, BP, pulse, weight, height, BMI and MUAC as applicable. **Nurse / Counsellor**

**IMMEDIATE ESCALATION OR
INFECTION-CONTROL ACTION
NEEDED?**

YES

NO / STABLE

ACT NOW

Emergency/unstable: fast-track immediate care.
Suspected infectious/outbreak-prone: apply IPC/PPE and separate the patient.

GUIDE CLIENT TO THE NEXT SERVICE POINT

- 7 Orient and guide the client to the correct next service point, supporting navigation through the clinic flow as needed. **CHW, Records staff or
Nurse**

Registration point minimums: paper/current electronic records access, patient card/booklet, calibrated vital-sign equipment and IPC supplies.

Monitoring Targets

Patients correctly registered:	100%
Records with complete locator information:	≥95%
Patients screened for priority chronic conditions:	≥95%
Recommended time to care:	≤30 minutes
Track waiting time to care and maintain accurate registration/screening documentation.	

Reference Materials

IPC Guidelines and SOPs
Patients' Rights and Responsibilities Charter
IDSR Guidelines



Purpose: Centralized appointment management to support timely patient review and continuity of care. **Applies to:** the point where the appointment register is stationed.

When: every patient encounter; scheduled and walk-in clients.

CONFIDENTIALITY: Reminders only for consenting clients; use neutral wording that does not reveal a condition.

BEFORE THE APPOINTMENT / START OF CLINIC DAY

REVIEW EXPECTED CLIENTS FOR THAT DAY & PREPARE FOR THEM

- 1 Review the Appointment Register/EMR; identify the integrated services due today and prepare/retrieve patient files before arrival.

Nurse + Data Clerk

SEND PRE-APPOINTMENT REMINDERS

- 2 Contact consenting clients by SMS/phone before the visit. Keep wording confidential and non-condition-specific. Lay providers may support reminders only when formally assigned.

Designated HW / Nurse

PATIENT ARRIVES / PRESENTS FOR CARE

SCHEDULED FOR TODAY?

YES

NO

3A SCHEDULED CLIENT

Nurse / Data Clerk

Confirm the appointment; update EMR/register. Use the all-services checklist to verify what is due today, then direct the client to the correct care station.

3B WALK-IN CLIENT

Nurse / Data Clerk

Record the walk-in; update the due-date page; document the reason (e.g., illness, refill shortage or missed visit), then direct the client to the appropriate care station.

CARE / SERVICES PROVIDED

PLAN THE NEXT VISIT PERIOD & SERVICES

- 4 Based on clinical assessment, determine the return period; record it in the patient card/booklet and/or EMR; document all integrated services due on the next visit and explain them to the client.

Clinician

CONFIRM & RECORD THE APPOINTMENT DATE IN THE APPOINTMENT REGISTER/EMR

- 5 Confirm the service package and enter the appointment date in the Appointment Register/EMR. Align the date with the documented care plan; use dispenser/pharmacy refill information where relevant.

Nurse / HW handling Appointments

CONDUCT END-OF-DAY RECONCILIATION & MISSED-VISIT HANDOVER

- 6 Reconcile expected clients versus actual arrivals; identify missed appointments; generate a list for the follow-up team (see SOP 13); update the Appointment Register/EMR before close of day. Handover the missed-appointment list for tracking.

Nurse + Data + Clinician

ROLE CLARITY: Clinician determines the clinical return period. Nurse/designated appointment HW schedules and records the date. They do not replace the clinician's clinical decision. Lay providers may support reminders/guidance around the clinic if designated.

Illustrative Monitoring Indicators & Outcomes

On-time appointment keeping: **>=90%** | Paper/EMR reconciliation: **100%**
Appointment Register documentation: **100%** | Missed appointments handed over daily to the patient tracking / follow-up team.
Outcome: clinic decongestion, adherence, continuity of care and improved treatment outcomes.

References & Implementation

Integration Standard 2 (Practical Guide for Integration)
Consolidated HIV/AIDS Guidelines Uganda (2022/2023).



Purpose: To provide accurate, culturally appropriate education and practical skills that strengthen self-care, disease prevention and coping. Applies to all facility and community service delivery points. **When?** Every patient encounter and planned group session.

CORE PRACTICE: RESPECTFUL, STIGMA-SENSITIVE, UNDERSTANDABLE EDUCATION WITH PROTECTION OF PATIENT PRIVACY AND DIGNITY

MAINTAIN A FUNCTIONAL PATIENT EDUCATION SCHEDULE

- 1 Keep a functional schedule showing topics, dates and responsible staff. Include services available at the health facility among the education topics.

In-Charge / Nurse
Lead Clinician

PREPARE THE SESSION, SPACE & MATERIALS

- 2 Protect time for education. Use a quiet, comfortable, clean space; gather IEC and demonstration materials plus the education register; prepare language-appropriate content and translated materials where available.

Nurse / Counsellor
Lead Clinician

WHICH EDUCATION
APPROACH FITS THE CLIENT /
GROUP TODAY?

GROUP

INDIVIDUAL

3A CONDUCT GROUP EDUCATION

Nurse / Counsellor / Clinician

Conduct daily group sessions in clinics, with special focus on youth, older persons, pregnant women, high-risk clients and other special groups. Provide tailored disease-specific information in a language clients/caregivers understand; use translated materials where available. Record the session in a log book.

3B CONDUCT INDIVIDUAL EDUCATION

Clinician / Nurse / Counsellor / Nutritionist

Build rapport; assess knowledge, literacy, beliefs and support needs. Identify barriers such as stigma, transport, side effects or forgetfulness. Demonstrate medicine use, diet, exercise and symptom monitoring; provide take-home IEC where available. Co-develop a SMART goal and self-management plan; involve a trusted support person only with consent.

DOCUMENT EDUCATION & THE AGREED SELF-MANAGEMENT PLAN

- 4 Document group or one-to-one education in the education register. Co-document the agreed self-management plan and SMART goals where applicable.

Responsible
provider

LINK TO ONGOING SUPPORT WHEN NEEDED

- 5 Link the client to peer groups, VSLAs or community programmes for community-based education, self-management support and follow-up when needed.

Counsellor / VHT /
Peer

IMPLEMENTATION ACTIONS

Clarify roles at each step; strengthen stigma-sensitive counselling; ensure IEC materials are used, and appropriate documentation registers are filled; make staff time for education; provide mentorship; integrate education into routine chronic-care visits. Essentials: education schedule, IEC/demonstration materials, education register and translated materials where available.

Monitoring Indicators & Outcome

Regular scheduled education (group or one-to-one): **>90%**
Education sessions implemented as scheduled: **100%**
Group + individual sessions documented: **100%**
Clients linked to self-management support as needed.
Outcome: Improved client/caregiver literacy and empowerment for self-care.

References

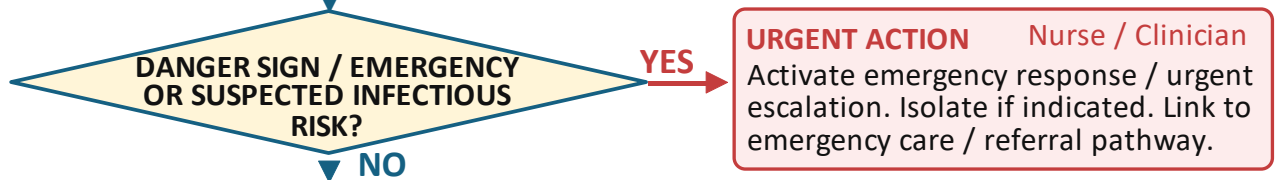
Relevant MoH health education materials; Counselling Job Aids; Flip Charts.

Purpose: Evaluate physical, mental and social well-being to support timely diagnosis and an individualized care plan. **Applies to** all facility/community service delivery points including outreaches. **When:** At every patient encounter as required.

START: CLIENT PRESENTS FOR CARE - ASSESS HOLISTICALLY AT EVERY ENCOUNTER

CONFIRM IDENTITY, CONSENT, TRIAGE & IPC

- 1 Confirm identity and obtain consent where necessary. Record vital signs; conduct a rapid screen for danger signs and outbreak-prone conditions. Apply IPC and isolate when indicated. **Nurse / Clinician**



TAKE A COMPREHENSIVE HISTORY & SCREEN HOLISTICALLY

- 2 Medical and family history; psychosocial needs/support; functional status/Activities of Daily Living (ADLs); self-management and lifestyle; medicines, allergies, adherence, interactions and side effects. Screen for coexisting conditions, lifestyle risks and preventive/supportive-care needs. **Clinician Nurse supports screening**

REVIEW SYMPTOMS, MONITORING RESULTS & DISEASE CONTROL

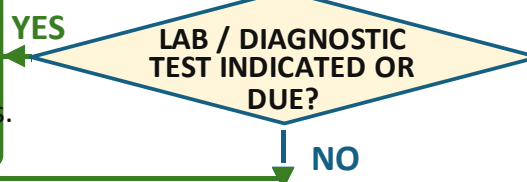
- 3 Review current symptoms, interval events, previous investigations and indicators such as viral load, blood pressure or blood sugar. Identify due or pending tests. **Clinician / Nurse**

CONDUCT A FOCUSED / THOROUGH PHYSICAL EXAMINATION

- 4 Identify new or worsening conditions, complications, functional limitations or treatment-related problems. **Clinician**

TEST + ACT ON RESULT

Clinician orders indicated test(s). Laboratory staff performs/reports. Clinician reviews and acts on results. **Clinician + Laboratory staff**



CATEGORIZE THE CLIENT & DEVELOP AN INDIVIDUALIZED CARE PLAN

- 5 Use national guidance. Elicit patient goals and preferences for shared decision-making. Develop the individualized care plan and apply the 5As / DSD planning approach where appropriate. **Clinician + Patient / Caregiver**

AGREE FOLLOW-UP, REFERRALS & SERVICE-DELIVERY APPROACH

- 6 Arrange referrals, consultations, laboratory follow-up and the most suitable differentiated service-delivery approach with the patient. **Clinician / Nurse**

DOCUMENT & HAND OVER THE UPDATED CARE PLAN

- 7 Record findings, decisions, counselling and the updated care plan in the client record, register and EMR. Use eAFYA / routine digital prompts where available; set the next appointment and tracking needs. **Clinician / Nurse Data Officer**

Monitoring Indicators & Outcomes

Holistic assessment using Chronic Care Services Guide: **>=95%**
 Paper / EMR records updated: **>=95%**
Outcomes: comprehensive integrated management and improved care / treatment outcomes.

References & Implementation

Uganda Clinical Guidelines (UCG); IPC Guidelines; Standard MoH Screening Tools; disease-specific protocols and policy guidelines.



SOP 05 - EMERGENCY AND CRISIS MANAGEMENT [ver.1.0]

Purpose: Ensure timely pre-hospital and hospital emergency response, stabilization, referral/transfer and continuity of care for acute crises arising during chronic care. **Scope:** OPD, emergency department and scene/pre-hospital response. **Use:** when an emergency or acute crisis is identified.

READINESS: protocols/checklists | call & dispatch | emergency kit + oxygen | referral contacts | incident log | backup paper/power | continuity options

IDENTIFY THE EMERGENCY OR ACUTE CRISIS

- 1 Recognize an acute emergency or deterioration at community or facility level. Activate emergency response pathway without delay. **Community First Aid Responder Health Worker**

ACTIVATE THE ALERT SYSTEM, GIVE FIRST AID & CALL / DISPATCH THE AMBULANCE

- 2 Activate the community/facility alert, provide first aid as appropriate, and coordinate ambulance call and dispatch for urgent response or transfer. **Community First Aid Responder Call / Dispatch Officer**

CONDUCT RAPID TRIAGE (UTAT), IDENTIFY LIFE-THREATENING PROBLEMS & ESCALATE

- 3 Use Urgent Triage Assessment and Treatment (UTAT) to rapidly identify life-threatening problems and escalate immediately as needed. **Triage Nurse Clinician**

STABILIZE IMMEDIATELY USING EMERGENCY PROTOCOLS

- 4 Use protocols for acute complications (hypoglycemia, hypertensive crisis, asthma, SCD crisis, seizures/status epilepticus). Give oxygen/emergency treatment as indicated and ensure emergency drugs/supplies are ready. **Emergency Nurse Clinician Medical Officer**

- 5 **AFTER STABILIZATION: ADMIT OR REFER / TRANSFER?**
Decision by Clinician

ADMIT

REFER

ADMIT / CONTINUE EMERGENCY CARE

- 6A If manageable at this facility, continue emergency treatment, monitoring and admission. Seek technical oversight as needed.

Clinician / Medical Officer / Emergency Nurse

REFER / TRANSFER SAFELY

- 6B Stabilize before transfer. Alert the receiving facility, organize ambulance referral, document actions and continue essential care during transportation.

Clinician / Medical Officer + Ambulance Paramedic

COMMUNICATE, DOCUMENT & ACTIVATE CONTINUITY PLAN IF SERVICES ARE DISRUPTED

- 7 Communicate with the patient and receiving facility; update records and the communication/incident log. If services are disrupted, activate continuity options: telehealth/phone hotline, alternate medicine pick-up, pre-packed MMD refills and have a backup paper document system/power system. **Records Officer / Clinician Facility In-Charge / Pharmacy**

CONDUCT AFTER-ACTION REVIEW + DRILLS / SIMULATION / MENTORSHIP

- 8 Review the emergency response, update plans and training needs, strengthen SOP awareness and preparedness, and use drills/simulation/mentorship to improve readiness. **Emergency Team Facility In-Charge**

Monitoring & Quality Targets

Emergency patients arriving by ambulance: **$\geq 80\%$**
Life-threatening cases accessing emergency-unit care within 1 hour: **$> 80\%$**
Emergency-unit mortality rate: **$< 3\%$**
Hypoxemia cases receiving oxygen: **100%**

Reference Materials

EMS Strategic Plan
Emergency treatment protocols
Uganda Clinical Guidelines (UCG)



Purpose: To guide clients to choose differentiated care tailored to individual needs, preferences and circumstances. **Applies to** all facility and community service delivery points.

When: Every client encounter and whenever care needs change.

Core Principle: 5As: ASSESS -> ADVISE -> ASSIST -> AGREE -> ARRANGE

CONDUCT COMPREHENSIVE ASSESSMENT + APPLY THE 5As

- 1 Assess medical and psychosocial needs and begin an individualized care plan using Assess, Advise, Assist, Agree and Arrange.

Clinician / Nurse / Counsellor

IDENTIFY WHAT THE CLIENT NEEDS

- 2 Confirm clinical status, barriers to care, support needs, preferences and risks to continuity of care.

Clinician / Nurse / Counsellor

EXPLAIN THE AVAILABLE DIFFERENTIATED CARE OPTIONS TO THE CLIENT

- 3 Use language the client understands. Explain locally available facility-based and community/alternative DSD service options.

Clinician / Nurse Counsellor

ADVISE ON THE MOST SUITABLE OPTION

- 4 Consider clinical stability, services required, distance, adherence, support needs and personal circumstances.

Clinician / Nurse Counsellor

AGREE THE INDIVIDUALIZED CARE MODEL & FOLLOW-UP SCHEDULE WITH THE CLIENT

- 5 Agree with the client on the care approach and follow-up schedule while maintaining privacy and confidentiality.

Provider + Patient

REASSESS
& ADAPT

6. WHICH AGREED DSD APPROACH?
Choose the option that fits the individualized plan

FACILITY-BASED APPROACH

- 6A Examples: Fast Track Drug Refill (FTDR), Facility-Based Groups (FBGs) and Facility-Based Individual Management (FBIM).

Clinician / Nurse / Counsellor

COMMUNITY / ALTERNATIVE MODEL

- 6B Examples: Community Drug Distribution Points (CDDP), CCLAD, alternative drug distribution points and MMD.

Provider + CHEW / lay provider as applicable

DOCUMENT, SYNCHRONIZE SERVICES & MONITOR

- 7 Record the selected model, support measures, lifestyle actions and review schedule in EMR/paper/client card. Align refills, clinical reviews, tests and follow-up across facility/community teams.

Clinician / Nurse / Counsellor
Records staff / CHEW/lay provider as applicable

REASSESS AT EVERY ENCOUNTER

- 8 Review whether the model still fits the client. If needs, risks or circumstances change, return to assessment and agree a different approach.

Clinician / Nurse Counsellor

Monitoring Indicators & Outcome

Completed 5As + individualized plan: **>=90%**

Eligible clients enrolled in suitable DSD models: **>=80%**

Outcome: Clients agree on tailored models of care that address their specific needs for improved outcomes.

Reference

Implementation Guide for Differentiated Service Delivery Models of HIV and TB Services in Uganda (2020).

Purpose: To standardize service packages at different stages of care under differentiated service delivery. **Use at:** initiation, routine refill, treatment monitoring and re-engagement in care.

Core cadres: Clinician, Nurse and Lay Provider.

CONFIRM DIAGNOSIS / REASON FOR CARE & ASSESS CURRENT STATUS

- 1 Confirm diagnosis; assess clinical condition, psychosocial needs, adherence and treatment response. At triage, identify any deterioration. Arrange due laboratory/monitoring tests and use results to guide care.

Clinician / Nurse
Laboratory staff for
ordered tests

2. WHAT IS THE CLIENT'S STAGE / CARE STATUS? Use national disease- specific criteria

NEWLY DIAGNOSED / INITIATING CARE

- A Give condition-specific health education. Start treatment at the timing required by the relevant protocol. Document care and schedule the first follow-up. Tailor support for children/adolescents, pregnant/breastfeeding clients & at-risk groups.

Clinician
/ Nurse

RETURNING AFTER INTERRUPTION / RE-ENGAGEMENT

- B Identify the client returning after loss to follow-up. Reassess clinical status and barriers to adherence/care. Update the management plan and appointments; restart or adjust therapy according to disease-management protocols.

Clinician
/ Nurse

NOT ESTABLISHED / UNCONTROLLED

- C Develop an individualized, closer follow-up schedule. Review adherence, symptoms and treatment response. Manage acute complications/deterioration promptly, refer for specialized care as needed & continue monitoring tests as appropriate.

Clinician
/ Nurse

ESTABLISHED / CLINICALLY STABLE

- D Confirm stability using disease-specific criteria. Offer the most appropriate DSD option(s): MMD (3-6 months), fast-track refill, CCLAD/CDDPs, facility-based groups (FBGs) or Community models as applicable. Align refills, clinical reviews and due laboratory monitoring. Continue psychosocial support and comprehensive clinical review every 6-12 months as recommended.

Clinician / Nurse
Lay Provider
supports eligible
community/follow-
up activities

ALL CARE PATHWAYS CONTINUE WITH ROUTINE DOCUMENTATION, COORDINATION & REASSESSMENT

DOCUMENT, COORDINATE & REASSESS CLIENT SITUATION AT EVERY ENCOUNTER

- 3 Record the selected care model/service package in the client record/HMIS. Ensure due clinical reviews, refills and laboratory monitoring are completed. Reassess treatment response, barriers and client preferences; change the DSD model when needs or circumstances change.

Clinician / Nurse
Records/HMIS support
staff, Lay Provider for
assigned follow-up

Monitoring Indicators & Outcomes

Eligible clients on multi-month dispensing: $\geq 80\%$
Adherence to treatment: $\geq 95\%$
On-time clinical / refill visits: $\geq 90\%$

Outcomes: Continuity of care, improved adherence and improved patient treatment outcomes.

Implementation Notes & Reference

Principle: Use national disease-specific criteria and individualize the DSD model as needs change. • Apply DSD principles across chronic conditions, not only HIV/TB.

Reference: National DSD and disease-specific guidelines.



Purpose: Ensure safe, accurate and rational prescribing of medicines to improve patient outcomes and reduce medication errors. **Applies to:** Authorized prescribers - Medical Officers, Clinical Officers, Dentists and authorized Nurse Prescribers where applicable.

CONFIRM PATIENT & CLINICAL INFORMATION

- 1 Confirm the correct patient. Record/check full name, age/date of birth, sex, patient ID, weight (especially for children) and address where applicable. Confirm diagnosis/provisional diagnosis and known allergies.

Prescriber

CHOOSE MEDICINE USING STANDARD GUIDANCE & SAFETY

- 2 Select treatment in line with the Uganda Clinical Guidelines and the Essential Medicines and Health Supplies List. Use generic names where possible. Align with eAFYA/routine prescribing tools and check available medicines at the facility. Avoid poly pharmacy and bad prescription practices e.g., Antibiotic misuse.

Prescriber

WRITE COMPLETE MEDICINE DETAILS

- 3 For each medicine include: generic name, strength, dose, route, frequency and duration of treatment e.g., Amoxicillin 500 mg orally every 8 hours for 5 days.

Prescriber

APPLY SAFE PRESCRIPTION-WRITING STANDARDS

- 4 Write clearly and legibly. Use standard units such as mg and mL. Avoid dangerous abbreviations. Keep the prescription evidence-based and consistent with national treatment guidance.

Prescriber

QUICK CHECK: EVERY PRESCRIPTION MUST INCLUDE

Patient information

- Full name
- Age/date of birth
- Sex
- Patient ID
- Weight, especially children
- Address, where applicable

Clinical information

- Diagnosis/provisional diagnosis
- Allergies, if known

Medicine details

- Generic name
- Strength
- Dose
- Route
- Frequency
- Duration

CONDUCT A FINAL COMPLETENESS & ACCOUNTABILITY CHECK

- 5 Before the prescription leaves the prescriber, confirm patient details, diagnosis/allergy information and all medicine fields are complete and within the prescriber's authorized scope of practice.

Prescriber

PRESCRIPTION COMPLETE - READY FOR DISPENSING

- 6 A clear, complete prescription is ready for the dispensing process. The prescribing SOP ends here; dispensing follows in the separate dispensing SOP (SOP 09).

Prescriber ->
Dispenser /
Pharmacy handover

Cross reference this SOP with any previously existing SOPs at the facility.

Illustrative Monitoring Indicators

% of prescriptions with all required patient and medicine details completed: $\geq 95\%$
% of prescriptions compliant with Uganda Clinical Guidelines / approved treatment guidance: $\geq 90\%$
% of paediatric prescriptions with patient weight documented and dose appropriate for weight: 100%
% of prescriptions with documented allergy status: $\geq 95\%$

Facility Support Tools

- Uganda Clinical Guidelines.
- Essential Medicines and Health Supplies List.
- eAFYA/routine prescribing tools where applicable.
- Quick-reference job aids



SOP 09 - DISPENSING MEDICATION

[ver.1.0]

Purpose: Ensure safe and effective patient-centred dispensing and use of medicines.

Tools: prescription/EMR, dispensing log, pill counter, labels/pictograms/leaflets, ADR / Adverse Events Following Immunization (AEFI) and Active Drug Safety Monitoring (aDSM) forms.

VERIFY THE PRESCRIPTION

- 1 Review the prescription versus medicine available in the store/pharmacy. Check patient age, weight and sex; medicine formulation, strength and dose against standard treatment guidance. Consider clinically relevant medicine interactions. **Dispenser**

APPROPRIATE & CLEAR?

YES

NO / IN DOUBT

STOP & CLARIFY

Consult the prescriber and resolve unclear, inappropriate or interacting therapy before dispensing.

Dispenser + Prescriber

SELECT ONE MEDICINE AT A TIME

Dispenser

- 2 Choose the correct item and package. Check generic name, form, strength, unit size and expiry; be aware of look-alike medicines. Prepare one item at a time.

CHECK QUALITY, COUNT & PACKAGE SAFELY

Dispenser

- 3 Inspect for damage/spoilage; never dispense poor-quality medicine. Report suspected quality problems through the required pharmacovigilance process. Count with a clean tray and spatula/spoon - never use bare hands. Package the correct quantity for one patient only; close the source container immediately and keep the drying sachet in the container.

LABEL THE PACKAGE COMPLETELY

Dispenser

- 4 Include patient name; age when needed; generic name; strength; dosage form; quantity; expiry; dose; and clear instructions on when, how much, how and for how long to take it. Use pictorial instructions where relevant.

QUICK SAFETY CHECK

Before handover, confirm:

- Correct patient
- Correct medicine
- Correct form / strength
- Valid expiry / good quality
- Correct quantity
- Complete label
- Interactions considered
- Counselling given
- Understanding confirmed
- Privacy protected

RECORD THE DISPENSING

- 5 Enter the medicine in the dispensing log (HMIS PHAR 003) or update the EMR. Complete all required fields before handing over the medicines. **Dispenser / Records-HMIS Support**

DISPENSE PRIVATELY - ONE PATIENT, ONE ITEM AT A TIME

- 6 Explain why to take the medicine; the route; when; how much; duration; food/medicine interactions; side effects and what action to take if they appear; and appropriate storage. Protect confidentiality, especially at busy dispensing points. **Dispenser**

USE TEACH-BACK & CLOSE THE ENCOUNTER

- 7 Ask the patient to repeat the instructions. Reinforce completion of the prescribed course, return on appointment dates for chronic care, and never share medicines. If an ADR/AEFI or other safety concern is identified, use the appropriate reporting process/form (NDA reporting platform). **Dispenser**

Illustrative Monitoring Indicators

% of medicines dispensed with complete and correct labels: 100%
% of dispensing transactions fully documented in HMIS PHAR 003 or EMR: 100%
Privacy and Confidentiality at the dispensing point
Medicines checked for expiry and quality before dispensing: 100%

Records & Safety Forms

- Prescription / EMR
- HMIS PHAR 003 dispensing log
- HMIS PHAR 011 ADR/AEFI form
- aDSM form
- Labels / pictograms / leaflets
- Pill counter / clean tray / spatula



SOP 10 - REFERRAL AND LINKAGE OF CLIENTS TO CARE

[ver.1.0]

Purpose: To ensure seamless, timely and structured bidirectional linkages within and between health facilities and community services, improving continuity of care, adherence, retention and monitoring of treatment outcomes.

KEEP REFERRAL INFORMATION READY

Clinic Team / Linkage Focal Person / CHEW / VHT

- 1 Maintain an up-to-date facility and community referral directory showing available services, contacts, operating hours and community support options.

IDENTIFY THE NEED FOR REFERRAL / LINKAGE

Clinic Team / Linkage Focal Person / CHEW / VHT

- 2 Identify clients needing another service or follow-up: new diagnosis, emergency/unstable, missed visit, psychosocial or other need. Refer on the same day the need is identified.

EMERGENCY / UNSTABLE?

YES

PRIORITIZE

Stabilize/first respond, alert the receiving point and arrange urgent transfer using the emergency/referral process.

Clinician / Nurse / Triage

NO / STABLE

CHOOSE PATHWAY & CONFIRM DESTINATION

- 3 Use the intra-facility, inter-facility or facility-community pathway. Confirm the receiving service is available before sending the client.

Clinician / Nurse /
Midwife
Linkage Focal Person

COMPLETE THE STANDARD REFERRAL FORM

- 4 Use triplicate referral form or EMR referral module. Record the reason, relevant findings and services required; obtain consent where needed and retain the originating record for tracking.

Referring Provider
Linkage Focal
Person / CHEW

TRIPPLICATE REFERRAL FORM

COPY 1 - TOP

Goes with the client to the receiving facility/point.

COPY 2 - MIDDLE

Returned to the referring facility/point as feedback on arrival and care.

COPY 3 - BOTTOM

Remains in the referral book for record-keeping, tracking and audit.

ALERT THE RECEIVING POINT & ARRANGE NAVIGATION

- 5 Contact the receiving service before referral. Give directions and appointment details, and arrange escort, accompaniment or transport support when needed.

Linkage Focal Person / Nurse / Clinician
CHEW / VHT / Peer / Nutritionist

RECEIVING PROVIDER RECEIVE THE CLIENT & PROVIDE THE REQUIRED SERVICE

- 6 Verify the referral, receive/register the client and provide the indicated clinical or non-clinical service without unnecessary delay.

Clinician / Nurse / Midwife
Lab Staff / Service-Point Provider

SEND FEEDBACK & CLOSE THE LOOP IN 7 DAYS

Provider / Linkage FP / VHT / CHEW / Peer / Counsellor

- 7 Send referral outcome to the referring source using the feedback copy, EMR/digital feedback, SMS or phone. Follow up within 7 days to confirm completion; trace or support clients who need navigation, transport or follow-up.

UPDATE RECORDS & CONTINUE CARE

Nurse / Records Officer / Clinician

- 8 Update the EMR, community-facility referral/linkage register and follow-up tools. Act on feedback, update the care plan and record the next appointment.

Monitoring & Quality Indicators

Referrals completed with documented feedback: **>=90%**
Referral directories with up-to-date contacts: **Current**
Community-identified clients successfully linked: **>=90%**

Safety & Quality

- Protect privacy and confidentiality.
- Use respectful, stigma-free communication.
- Prioritize emergency referrals.
- Confirm service availability before sending.
- Ensure the client knows where to go and what happens next.



SOP 11 - LINKAGES TO PATIENT SOCIOECONOMIC EMPOWERMENT [ver.1.0]

Purpose: Link vulnerable clients to social-support and empowerment structures that reduce vulnerability, strengthen retention in care and improve treatment outcomes. **Examples:** VSLAs, PDM/SACCOs, youth groups, adherence groups and other community support services.

MAP SUPPORT SERVICES & IDENTIFY VULNERABLE CLIENTS

- 1 Maintain a current directory of community groups, livelihood programmes, PDM/SACCO contacts and social-welfare actors. Identify clients who may be vulnerable during consultation, adherence counselling, missed-appointment tracking or outreach.

Facility Team / Counsellor
CHEW / VHT / Clinician
Nurse / Parasocial Worker

ASSESS CLIENT NEEDS & OBTAIN CONSENT

- 2 Conduct socioeconomic/vulnerability assessment. Discuss the proposed support and obtain consent before referral or linkage.

Clinician / Nurse
Counsellor

MATCH THE CLIENT TO APPROPRIATE SUPPORT

- 3 Choose support that fits the client need: community support group, PDM/SACCO, VSLA, ART/adherence group, youth group, mentor-mother group, NCD group or another mapped social service.

Counsellor
Peer
Educator

REFER / LINK & DOCUMENT THE REFERRAL

- 4 Link the client to the selected community group, PDM/SACCO, VSLA or social service. Record the referral in the linkage register using the minimum documentation package and name the person responsible for follow-up.

Counsellor /
Peer Educator
Health Worker

SUPPORT NAVIGATION WHEN NEEDED

- 5 For vulnerable clients who cannot complete linkage alone, arrange directions, escorted linkage, transport support or phone contact according to available resources.

CHEW / VHT / Peer
Counsellor / Partner

CONFIRM SUCCESSFUL LINKAGE WITHIN 2-4 WEEKS

- 6 Confirm with the client, CHW/VHT, PDM focal person, group leader or receiving service that linkage occurred. Update the record and re-link the client if the first referral failed.

Clinician / Nurse /
Counsellor
CHEW / VHT

FOLLOW UP WELLBEING, ADHERENCE & RETENTION

- 7 During routine visits, review whether the support is helping and monitor adherence, wellbeing and retention in care. Escalate high-risk clinical or psychosocial concerns.

Clinician / Nurse
Counsellor

SUPPORT GROUP ACTIVITIES, DOCUMENT & REPORT

- 8 Mobilize members, track attendance and missing members; follow up high-risk cases. Conduct structured psychosocial, adherence, disclosure, lifestyle and peer-learning sessions weekly/bi-weekly. Maintain linkage registers and include summaries in facility reports.

Group Leader
Counsellor / Peer /
VHT

Monitoring Indicators

Patients screened for vulnerability:
Patients referred to PDM/community groups:
Successful linkage rate:
Retention in care:
Treatment adherence outcomes:

>90%
Track
>90%
>90%
Track

Reference Materials

- Parish Development Model Implementation Guidelines
- Differentiated Service Delivery Guidelines
- Health Sector Development Plan
- Community Health Strategy



Purpose: To standardize integrated outreach services that improve access, re-engage missed clients and strengthen facility-community linkage. **Applies to** satellite sites, mobile clinics, outreach camps, CDDPs and other community points. No outreach should be standalone - services should be integrated & coordinated.

PLAN & PRIORITIZE THE INTEGRATED OUTREACH

- 1 Develop the quarterly integrated outreach plan aligned with facility targets and community needs. Allocate resources, coordinate partners, then use facility/community data to prioritize hard-to-reach areas, high-burden areas, service gaps and missed clients.

Facility In-Charge
/ Outreach Lead
VHTs / CHEWs

MOBILIZE THE COMMUNITY

- 2 Use local leaders, radio, VHT networks, peer groups and other approved channels to inform priority populations about the outreach.

VHTs / Peers
Community
Leaders

SET UP A SAFE, PRIVATE OUTREACH SITE

- 3 Organize client flow, IPC, privacy/confidentiality, supplies, registers and service stations before service delivery starts.

Outreach Team

TEAM READINESS CHECK

- ☐ Approved integrated outreach plan
- ☐ Staff/cadre roles assigned
- ☐ Transport & allowances arranged
- ☐ Medicines/supplies available
- ☐ Registers/digital tools ready
- ☐ Referral contacts/directory ready
- ☐ Post-outreach follow-up team/individual named

PROVIDE INTEGRATED COMMUNITY SERVICES

- 4 Provide health education, screening, refills, adherence counselling, psychosocial support, laboratory sample collection and referrals, together with relevant preventive, promotive and environmental health services.

Clinician / Nurse /
Counsellor
Health Assistant /
Inspector

RE-ENGAGE MISSED / LOST-TO-FOLLOW-UP CLIENTS

- 5 Identify clients found during outreach or tracing activities; document re-engagement and schedule the next appointment or service contact.

Clinician / Nurse /
Records Officer

REFER COMPLEX CASES OR EMERGENCIES

- 6 Use standardized referral forms and link the client to the nearest appropriate facility. Prioritize urgent cases and confirm with the referral destination.

Clinician / Nurse

DOCUMENT SERVICES & RECONCILE DATA

- 7 Record all services in outreach registers and reconcile with facility EMR/records within 24-48 hours. Keep documentation simple and avoid duplicate parallel tools.

Records Officer
Data Team

DEBRIEF & ASSIGN POST-OUTREACH FOLLOW-UP

- 8 Review performance, challenges and improvement actions. Assign responsibility for following referrals, missed clients, pending laboratory results and other unresolved needs after the outreach. Report work done.

Outreach Team Lead
Designated follow-up
staff

IMPLEMENTATION PRINCIPLE: maintain cross-cadre teamwork; plan transport, allowances and supplies; use simple integrated documentation; and name the person who owns post-outreach follow-up.

Illustrative Monitoring Indicators

Outreach sessions conducted as planned: **>=90%**
Clients screened for priority conditions: **>=95%**
Outreach data reconciled within 48h: **100%**
Identified missed clients re-engaged: **>=85%**
Referrals with documented feedback: **>=90%**

Outcome & Reference

Outcome: improved access to integrated services, increased re-engagement and stronger community-facility linkage. **Reference:** Relevant MoH community and outreach guidance.

Timing: as per approved outreach plan, targeted tracing or service-gap response.



SOP 13 - PATIENT TRACKING AND FOLLOW-UP

[ver.1.0]

Purpose: To ensure timely follow-up, reduce missed appointments & loss to follow-up, and strengthen continuity of care through facility-community tracking. **Key terms:** Missed appointment = failure to return on scheduled date, usually >7 days. LTFU = absent >28 days for HIV or >60 days for NCDs.

REVIEW EXPECTED VS ACTUAL ATTENDANCE DAILY

- 1 Compare the daily appointment list with actual attendance and identify missed appointments promptly. Nurse / Data Clerk/ Designated staff

GENERATE THE MISSED-APPOINTMENT LIST

- 2 Create the daily line list of clients who missed appointments and verify available locator/contact information before tracing. Nurse / Data Clerk

INITIATE FOLLOW-UP WITHIN 24-48 HOURS OF A MISSED VISIT

- 3 Use phone call, SMS or approved community/home follow-up (using locator data) where there is no phone access. Keep a phone/contact log as evidence of follow-up. Protect confidentiality- do not reveal the client's condition in messages. Record unsuccessful or wrong-number attempts and resort to alternative follow-up method. HW / Nurse / VHT / CHEW / Linkage / Expert Client

DOCUMENT THE TRACKING OUTCOME

- 4 Record one outcome: reached/returning; reached/not returning (record reason); not reachable; or wrong/unreliable number. If the traced client is available, schedule a new appointment. Tracking staff Nurse / Data Clerk

IF STILL NOT RE-ENGAGED, INTENSIFY TRACING

- 5 For clients who remain missed/LTFU, use community systems such as VHT or peer follow-up. Update the status in the EMR or patient appointment register. VHT / CHEW / Peer / Linkage Facilitator/ Designated HW

WHEN THE CLIENT RETURNS, RE-ENGAGE IN CARE

- 6 Counsel on adherence and continuity of care; identify root causes of poor appointment keeping; discuss clinical risks and involve the client in solutions. Clinician / Nurse Counsellor

REASSESS CLINICAL STATUS & UPDATE CARE PLAN

- 7 Conduct comprehensive clinical assessment and relevant laboratory tests as needed. Update treatment/care plan, appointment, client notes, EMR and registers. Clinician / Nurse Lab staff as indicated Records/Data staff

REVIEW TRACKING PERFORMANCE ROUTINELY

- 8 Review missed appointment tracking and the tracking outcomes weekly; review adherence/retention monthly with the clinical team and use findings for quality improvement. Facility clinical/ data team In-Charge

FOLLOW-UP PRINCIPLE: apply tracking to all chronic conditions and keep facility-community records aligned.

Monitoring Targets

Daily paper/EMR reconciliation:	100%
Weekly tracking outcomes documented:	100%
Follow-up within 24-48 hours:	100%
Adherence/retention review:	Monthly

Outcomes & Reference

Outcomes: improved adherence, treatment outcomes, longitudinal follow-up and continuity of care.

Reference: Consolidated Guidelines for Prevention and Treatment of HIV & AIDS in Uganda (2022/2023).



SOP 14 - HOW TO PREPARE AND CONDUCT CPD SESSIONS

[ver.1.0]

Purpose: Institutionalize continuous capacity development so health workers remain knowledgeable, competent and able to provide quality care across all departments, service points & health-worker cadres.

IDENTIFY STAFF TRAINING NEEDS In-Charge / Dept Head / CME Coordinator / QI Focal Person

- 1 Review performance data, audits, supportive-supervision findings, service-delivery gaps, CQI priorities and emerging needs. Identify the problem and likely root causes.

PREPARE THE QUARTERLY CPD/CME PLAN CME Coordinator / Facility In-Charge

- 2 Develop a CPD schedule showing topics, date/frequency, target staff, presenter, learning method and responsible person. Align topics with learning gaps, post-mentorship recommendations and CQI priorities.

PREPARE THE SESSION CME Coordinator / Designated Presenter

- 3 Confirm the facilitator and venue; notify staff in advance; prepare agenda, learning materials, attendance register, feedback/pre-post tools and demonstration supplies where needed.

CONDUCT THE CPD/CME SESSION Designated Presenter / Facilitator

- 4 Register attendance. Use interactive methods such as slides, flip charts, case discussions or practical demonstrations. Summarize key learning points/action items and share materials.

CONNECT LEARNING TO PRACTICE & CQI Presenter / QI Focal Person / Department Head

- 5 Link the session to a current CQI problem, supervision finding or service gap. Include QI-project learning where relevant. Agree with what staff should do differently after the session and who will follow up.

DOCUMENT THE SESSION CME Coordinator / QI Focal Person

- 6 Maintain the training/attendance register, topic and facilitator record, materials/presentations, pre/post-test results, mentorship logs and QI documentation as applicable. Record CPD points according to the applicable system or professional registration body.

COLLECT FEEDBACK & CHECK IMPACT CME Coordinator / QI Focal Person / Department Head

- 7 Collect participant feedback and identify unresolved learning gaps. Review application of learning through post-training checks, mentorship, audits, QI indicators or supportive supervision.

REPORT, IMPROVE & PLAN THE NEXT SESSION CME Coordinator / Facility In-Charge / QI FP

- 8 Include CPD/CME activities in facility reports. Review what worked, barriers and priority gaps; update the next quarterly schedule and arrange mentorship/support where required.

CPD PRACTICE PRINCIPLE: learning should respond to identified service gaps, use interactive methods, be documented, and be followed by checks that staff apply learning in practice.

Monitoring & Quality Indicators

% of planned CPDs conducted weekly: **>=90%**
% of clinical staff trained: **>=90%**
Feedback & follow-up actions: **Documented & Completed**

Expected result: a knowledgeable, competent workforce applying learning to service quality and patient care.

Documentation & Reporting

- CPD/training register: attendance, topics, facilitators and CPD points
- Feedback/evaluation forms & action logs; meeting minutes
- Training materials/presentations; pre/post-training checks
- Mentorship logs, QI documentation journals/run charts/project tracker



SOP 15 - ROUTINE CLIENT SATISFACTION FEEDBACK

[ver.1.0]

Purpose: To continuously document, measure and improve client satisfaction through routine feedback and timely response. **Applies to** all facility and community service points.

MAKE FEEDBACK OPTIONS VISIBLE & ACCESSIBLE

Facility In-Charge / Service-Point Lead

- 1 Display approved feedback channels at accessible service points: QR code, suggestion box, paper-based method and other approved options.

ORIENT THE CLIENT - PARTICIPATION IS VOLUNTARY

- 2 Briefly explain how to give feedback. Make it clear that participation is voluntary and will not affect the client's care.

Frontline Staff / VHT / Peer Educator / CHEW

OFFER ASSISTED / ALTERNATIVE FEEDBACK WHEN NEEDED

- 3 If a client cannot use the QR option, offer an approved paper, interviewer-assisted or simple phone-based method. Assistance should support access without bias

Front Desk /
Social Worker
Peer / CHEW /
VHT

USE MORE THAN ONE FEEDBACK CHANNEL WHERE POSSIBLE

- QR code/Digital tool
- Suggestion box
- Paper feedback
- Interviewer-assisted feedback without bias
- Simple phone-based methods

PROTECT ANONYMITY, PRIVACY & INDEPENDENCE

- 4 Do not request names or identifiers unless required by the approved tool. Never manipulate, coerce, suppress or discourage negative feedback.

All Staff

REVIEW FEEDBACK FOR URGENT ISSUES & TRENDS

WIT / Facility-in-charge

- 5 Review the dashboard/reports daily for urgent concerns and routinely/monthly for recurring themes, service gaps and action points.

DOCUMENT FINDINGS & AGREE CORRECTIVE ACTIONS

WIT / Facility-in-charge

- 6 Use quality-improvement methods to define corrective actions, responsible persons and timelines. Update the approved/digital action tracker. Initiate QI projects as needed.

IMPLEMENT, TRACK & REVIEW ACTIONS

WIT / Action owner

- 7 Follow corrective actions to completion and review whether they improved the identified client-experience problem. Escalate unresolved issues through appropriate facility structures.

SHARE RESULTS & CLOSE THE FEEDBACK LOOP

WIT /Data Officer/ Facility-in-charge

- 8 Share findings and agreed actions during performance reviews, QI and HUMC meetings and relevant forums. Inform clients/communities about actions taken through health talks, community meetings, radio or other approved channels.

FEEDBACK PRINCIPLE: Participation is voluntary; protect anonymity, privacy and independence; routine collection must lead to timely action and visible response.

Monitoring Targets

Clients submitting feedback: **vs monthly facility target**
Actions documented & completed in digital action tracker: **per quarter**
Community feedback sessions conducted: **quarterly**

Outcome: client feedback informs quality-improvement projects at the health facility and community.

Routine expectation: review urgent concerns daily, identify recurring themes, assign actions and close the loop with clients/communities.

Reference Materials

MoH Routine Client Satisfaction Feedback Implementation Guidelines (2024)

Use approved facility feedback tools, action trackers and communication channels.

Purpose: To promote complete, accurate, secure and timely data use for decision-making and service improvement. **Applies to** Paper-based and electronic systems at facility and community levels.

RECORD DATA AT THE POINT OF CARE

- Record every visit, treatment update, laboratory result, referral and appointment in the required source tool at the time care is provided. **All health workers**



ENTER / UPDATE ELECTRONIC SYSTEM

Enter/update the approved EMR/electronic system & keep the required source record aligned.

Point-of-care staff / Records support

DOWNTIME - CONTINUE ON PAPER

Use approved paper tools. Enter/reconcile with the EMR when service is restored.

Point-of-care staff / Records / IT support

CHECK COMPLETENESS & CORRECT ERRORS ROUTINELY

Records Team / QI Team

- Review records for missing fields, inconsistent values, duplicate entries and obvious errors; correct promptly using approved source records.

CONDUCT MONTHLY DATA AUDIT & RECONCILIATION

Records & QI Teams

- Reconcile paper tools & the EMR + any other source documents. Document discrepancies, initiate and document corrective actions.

ESCALATE UNRESOLVED DATA / SYSTEM PROBLEMS

Records Lead / In-Charge / IT staff

- Notify the supervisor/records lead and technical/IT support staff when tools, access, system instability or unresolved discrepancies prevent accurate reporting. Track to resolution.

ANALYSE APPROVED INDICATORS & PERFORMANCE TRENDS

Clinical / Records QI Team

- Review approved indicators such as retention, control rates, complications and missed appointments; identify gaps requiring action.

HOLD PERIODIC PERFORMANCE REVIEW & AGREE ACTIONS

- At least quarterly, review findings, identify gaps, assign responsible persons and timelines, and link priority gaps to QI actions. **Facility Team / QI Team / In-Charge**

TRACK AGREED ACTIONS TO COMPLETION

Action Owner / QI Team / Facility In-Charge

- Follow agreed actions with responsible technical teams until completed and document implementation progress and results.

PROTECT, STORE & BACK UP DATA

SI / Records Team / Facility Management

- Maintain role-based access, secure paper-record storage, confidentiality and routine backup procedures for electronic data.

DATA QUALITY PRINCIPLE: document at source, reconcile routinely, correct promptly, use approved indicators and protect confidentiality.

Monitoring Targets

Paper and EMR completeness:	≥95%
Paper-EMR alignment:	≥95%
Weekly missed appointment lists generated:	100%
Monthly reviews conducted:	100%
QI actions completed:	≥80%

Outcome: improved data quality and information use for decision-making and performance improvement.

Reference Materials

National HMIS

Use MoH DHI- approved indicator definitions and reporting tools applicable to the service area.



Purpose: Continuously identify and address health-service delivery gaps.

Applies to all health service providers at facility and community levels.

ESTABLISH AND FUNCTIONALIZE QI TEAMS

- 1 Establish the Facility QIC, Departmental QI teams and WITs. Facility in-charge
Ensure each team is functional and has a regular meeting schedule. Department or Unit QI leads

COMMUNICATE ROLES & SET TIME FOR CQI MEETINGS & PROJECTS

- 2 Communicate QI roles & responsibilities to unit heads and team members. Facility in-charge
Set regular meeting schedule (monthly for facility/dept teams; weekly for WITs) Department / Unit Heads
WIT leads

REVIEW DATA TO IDENTIFY PERFORMANCE GAPS

- 3 Use observations of care processes, Case-management reports, Chart/Data reviews and available tools such as RSCFI, Audit tools and DHIS2 data to identify and prioritize performance gaps. Facility in-charge
Department Head
Unit Head
QI Team
WIT

SIMPLE CQI CYCLE

- 1. Find the gap**
Use data and observations.
- 2. Find the cause**
Conduct RCA.
- 3. Test improvement**
Implement the action plan.
- 4. Measure change**
Track KPI's/run chart.
- 5. Sustain & spread**
Standardize what works.

CONDUCT ROOT CAUSE ANALYSIS (RCA)

- 4 For each priority gap, use an RCA method to identify the primary cause(s) before choosing solutions. Facility in-charge
Record the key causes identified. Department Head
Unit Head
QI Team
WIT

DEFINE THE IMPROVEMENT AIM AND DEVELOP A QI ACTION PLAN

- 5 Develop a SMART improvement aim for the priority problem. Identify at least one or two QI project(s) to address it. Agree the changes to test, responsible persons and timelines. Facility /
Department in-charges
Department heads generate QI projects & supervise WITs. **WITs** carry out day-to-day improvement steps. **Facility QI Committee** checks department CQI performance. WIT leads

DOCUMENT AND MONITOR PROGRESS IN A DOCUM. JOURNAL

- 6 Open a QI documentation journal for each project. Update progress, run charts and the QI database; review whether the relevant KPI is changing in the desired direction. QI lead
QI Committee
Department head
WIT lead

SUSTAIN, STANDARDIZE & RECOGNIZE IMPROVEMENT

- 7 When improvement is demonstrated, agree how the successful change will be maintained in routine work. Recognize team contributions and identify any support needed to sustain the gain. QI lead
QI Committee
Department head
WIT lead

SHARE RESULTS & START THE NEXT IMPROVEMENT CYCLE

- 8 Share results in WIT, department, facility and performance-review meetings. QI lead
Document best practices, lessons and remaining gaps, then select the next priority for improvement. QI Committee
Department head
WIT lead

Monitoring Indicators

No of CMEs initiated to address an identified quality gap:	Once a quarter
Quarterly community feedback sessions conducted:	Once a quarter
% of facilities in the district with a functional facility/department QIC and WITs:	65%
% of QI project with demonstratable improvement	60%
Best practices documented and shared:	3 per

Reference Materials

National QI Framework and strategic Plan-NQIF & SP 2021-2025 Pg 30-32